

CONSTIPATION

Give urgent attention to the patient with constipation and:

- No stools or flatus/wind in the past 24 hours with abdominal pain/distension

Refer same day.

Approach to the patient with constipation not needing urgent attention:

- Review diet, fluid intake and medication (amitriptyline, schizophrenia treatment, codeine and morphine can cause constipation: discuss with doctor). Ask about regular use of enemas or laxatives.
- Exclude pregnancy ↗ 157. If pregnant, advise that constipation is common during pregnancy and give advice as below.
- If weakness/tiredness, weight gain, low mood, dry skin or cold intolerance, check TSH. If abnormal, refer to doctor.
- If patient is bed-bound or has a life-limiting illness, also consider giving palliative care ↗ 170.
- If > 65 years, bed-bound or receiving palliative care, check for solid immobile bulk of stool in rectum. If present, impaction likely: gently remove stool using lubrication. If unsuccessful, refer.
- Advise a high fibre diet (vegetables, fruit, coarse meal, bran and cooked dried prunes), adequate fluid intake and at least 30 minutes moderate exercise (e.g. brisk walking) most days of the week.
- If no better with diet and exercise, give **sennosides A and B** 13.5mg at night or **lactulose** 10-20 mL once or twice daily.
- If no response after 1 week of laxative use, or if recent change in bowel habits, weight loss, blood in stool or occult blood positive, or cause uncertain, refer.

ANAL SYMPTOMS

Give urgent attention to the patient with anal symptoms and any of:

- Extremely painful lump on anus
- Unable to pass stool because of anal symptoms

Refer same day.

Approach to the patient with anal symptoms not needing urgent attention

- If patient has anal sex, ask about genital symptoms and treat partner ↗ 49. If painless bleeding, passing mucus or unable to pass stools despite feeling the need to (tenesmus), treat for sexually transmitted **proctitis**: give single dose **ceftriaxone** 250mg IM¹ and **azithromycin** 1g orally. If severe penicillin allergy², omit ceftriaxone and increase **azithromycin** to 2g. If diarrhoea ↗ 46.
- Then examine anal area to look for cause:

Crack/s

If constipated, also advise and treat as above.

Apply **bismuth subgallate compound** ointment 6-12 hourly or **lidocaine 2%** cream before and after each bowel action.

Lump/pile

- Advise and treat as for constipation above, and advise to avoid straining.
- If pile cannot be reduced or is thrombosed, refer.

Ulcer/s or perianal wart/s

Treat as for genital ulcer →49.

Red/raw skin

- Advise good hygiene.
- Look for contact cause. If diarrhoea ↗ 46.
- Wash with **aqueous cream** (UEA), avoid soap.
- Apply **zinc and castor oil** ointment to raw areas. If severe itching, also apply **hydrocortisone 1%** cream twice a day for 5 days.

Suspected worms

- If tapeworm: give **albendazole** 400mg daily for 3 days. If other worm or unsure: give single dose **mebendazole** 500mg.
- Educate on personal hygiene and advise to avoid undercooked meat.
- Treat household members at the same time.

If no better with treatment, refer.

¹Dissolve **ceftriaxone** 250mg in 0.9mL **lidocaine 1%** without epinephrine (adrenaline). ²History of anaphylaxis, urticaria or angioedema.